NCCP 00254 · VERSION 5

Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Anastrozole: material risks

☐ Joint and muscle pain or stiffness

☐ Vaginal dryness and sexual symptoms

☐ Lipid / cardiovascular risk changes

☐ Hot flushes, sleep disturbance and fatigue

☐ Bone thinning and fractures

☐ Uncommon liver abnormalities or serious allergic reactions

A functioning premenopausal ovarian state is not compatible with AI monotherapy. Severe hepatic impairment is not recommended by NCCP; moderate impairment requires caution. Confirm menopausal status rather than assuming that absent periods after chemotherapy mean permanent menopause. Any ovarian-suppression treatment must have its own prescribed schedule.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile. Confirm menopause; hormone tests may be needed if it is uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Assess symptoms, adherence and bone health at reviews. Repeat lipids and other blood tests when clinically indicated. Bone-density timing follows baseline results and fracture risk.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending